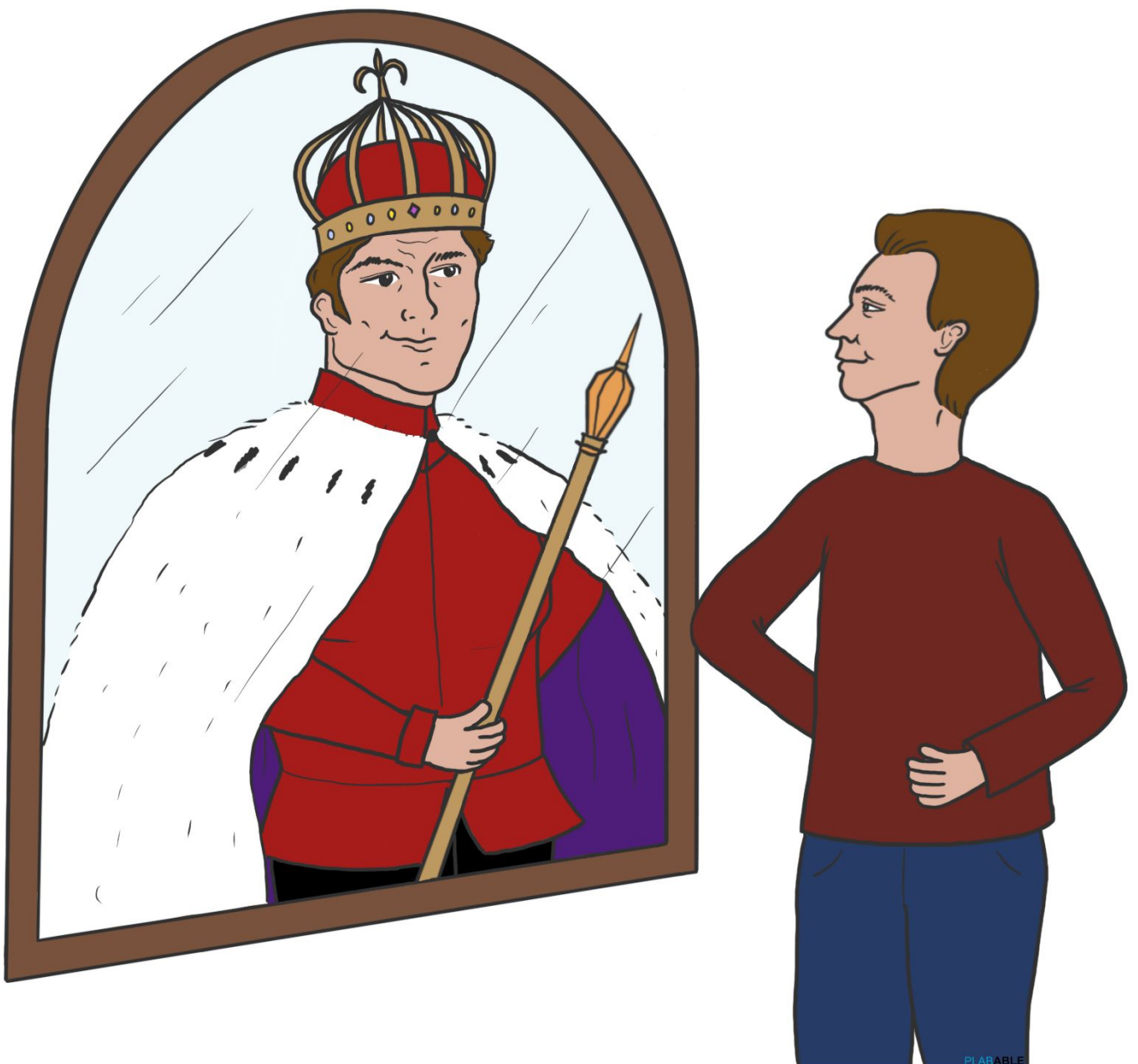


PLABABLE

GEMS

VERSION 1.2

PSYCHIATRY



Postpartum Mental Health Issues

Postpartum Psychosis	Postpartum Depression	Postpartum Blue
● Thoughts of harming baby ● Hearing voices saying baby is evil ● Hallucination ● Delusion ● Thoughts of suicide	● Low mood ● Loss of pleasure ● Anxiety ● Disturbed sleep ● Loss of appetite ● Loss of concentration ● Low self esteem ● Low energy levels ● Low libido ● Suicidal thoughts Can progress to depression	● Anxious ● Tearful ● Irritable ● Benign
Begins 2-4 days postpartum and peaks at 2 weeks	Seen 3-4 weeks following childbirth	Typically seen 3 -7 days following childbirth
● Admission in hospital ● Anti-psychotics ● Mood stabilizers ● ECT	● CBT ● SSRIs (Sertraline)	● Reassure

Somatic Symptom & Related Disorders

Somatic symptom disorder:

- Multiple symptoms involving multiple systems
- Chronic course
- History of excessive diagnostic testings
- Rejection of previous physicians

Treatment: Psychotherapy with regular visits to the same physician

Hypochondriac or illness anxiety disorder

- Constant thinking of acquiring or having a serious illness (Cancer, HIV) despite medical evaluation and reassurance
- Minimal to no somatic symptoms

Treatment: Cognitive behavioral therapy

Examples for hypochondriac disorder

- A person believing that a simple headache is because of a cancer despite all investigations
- A person thinking tiredness is caused by HIV
- A person believing that a benign lump is a cancer despite all investigations

Somatic Symptom & Related Disorders

Munchausen's syndrome

- Intentional production / falsification / fabrication of physical or psychological signs and symptoms to obtain medical attention and treatment

Example for Munchausen's syndrome

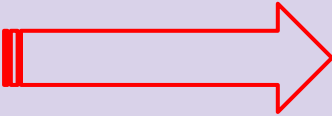
A woman always presents to the hospital with complaints of abdominal pain and bloody stools. She brings a sample from home which contains blood but is not able to produce a stool sample in the hospital. This is the 4th time she has visited the hospital. Her abdomen shows multiple scars of laparoscopic surgeries. She insists on getting more investigations.

Münchhausen's syndrome by proxy

Parent or carer produces factitious illness in a child or adult in their care

Somatic Symptom & Related Disorders

Conversion disorder:

- Loss of motor or sensory function:
 - Paralysis
 - Blindness
 - Mutism
 - Often triggered by a stressor
 - Patient do not intentionally fabricate the symptoms
- 

without organic cause

Examples:

- A 25 year old female was brought to the hospital with limb paralysis that developed after she witnessed a car accident. All the examinations are normal.
- A woman was brought to the hospital saying that she is unable to see after witnessing the death of her husband. All the examinations are normal.

Anorexia Nervosa

- Significant weight loss due to food restrictions, excessive exercise, self-induced vomiting, laxative or diuretic usage
- Fear of getting obese even though the patient is thin
- Calluses on the hands due to induced vomiting
- **BMI < 17.5**

Management

- Refer to medical ward for stabilisation with any BMI if:
 - Hypoglycemia
 - Bradycardia
 - Electrolyte imbalance - Hypokalemia
 - Signs of organ failure
- BMI <15 without medical complications refer urgently to **eating disorder unit**
- BMI 15 - 17.5 without medical complications refer to **eating disorder unit** or local community mental health team
- **Cognitive behavioral therapy**

Bulimia Nervosa

- Recurrent episodes of binge eating
- Compensatory mechanism to lose weight:
 - Fasting
 - Excessive exercise
 - Self-induced vomiting
 - Laxative or diuretics abuse
- Parotid swelling
- Thickened calluses on hand due to induced vomiting
- **BMI >17.5**

Treatment

- Cognitive behavioral therapy

Autism Spectrum Disorder

Features

- Developmental impairment of social and communication skills
- Stereotyped and repetitive behavior
- Preference for only specific hobbies (collecting toys)

Management

- Behavioral and development programmes
- Speech and language therapy

Asperger's syndrome:

- Also, known as high functioning autism
- IQ is average or above average in most cases
- Difficulty with socializing

Attention Deficit Hyperactivity Disorder (ADHD)

Features

- Hyperactivity, impulsivity, and/or inattention
- Symptoms present in ≥ 2 settings (eg, school, home) before 12 years of age
- Reluctant to engage in mentally-challenging tasks
- Excessive talking
- Easily distracted
- Does not seem to listen when spoken directly to
- Failing to finish schoolworks and chores

Management

Children:

- Methylphenidate (first-line)
- ADHD-focused group parent-training programme

Adults:

- Lisdexamfetamine or methylphenidate first-line in adults
- Cognitive behavioral therapy (CBT)

Attention Deficit Hyperactivity Disorder (ADHD)

Brain trainer:

A 6 year old boy was brought to the hospital saying that he is easily distracted and interrupts other students in the school. He is also careless, unable to stay on a task for a period of time and is unable to play quietly. What is the most likely diagnosis?

➔ **Attention deficit hyperactivity disorder**

Manic Disorder

Mania

- Elevated mood and increase in quantity and speed of physical and mental activity ≥ 1 week
- Diagnosis requires severe functional impairment or hospitalization
- Grandiose ideas
- Excessive amounts of energy
- Racing thoughts and flight of ideas
- Overactivity
- Needing little sleep
- Easily distracted

Hypomania

- Milder version of mania
- Does not cause marked functional impairment or need hospitalisation

Flight of Ideas

Brain trainer:

A patient describes the sensation of having an increased turnover of thoughts in his mind. What is the most likely thought disorder?

❖ **Flight of ideas**

Depression

One of the following:

- Constant low mood
- Loss of interest or pleasure in most activities

Plus three of the following:

- Fatigue
- Worthlessness
- Suicidal thoughts
- Decreased concentration
- Insomnia
- Weight loss

Symptoms should be there for > 2 weeks

Management:

- Antidepressants - **SSRIs** (first-line)
 - Fluoxetine
 - Sertraline
 - Citalopram
- **CBT**

Note:

- Drugs take 2-4 weeks to act
- Medication to be continued for at least 6 months

Dysthymia: Chronic mild depression for > 2 years

Depression

Brain trainer:

A woman presents with low mood, increased appetite, weight gain, somnolence and hypersomnia. What is the SINGLE most likely diagnosis?

→ **Atypical depression**

Adjustment Disorder

Brain trainer:

A woman has been feeling low and having difficulty in concentrating since her mother passed away 2 months ago. She feels lethargic and has been crying more often lately. What is the SINGLE most likely diagnosis?

→ **Adjustment disorder**

Bipolar Disorder

- Mania alternating with depression
- High suicide risk
- **Treatment:**
 - Mood stabilisers - **Lithium**
 - Cognitive behavioral therapy

Example:

A 40 year old female has recently spent a huge amount of money on buying new accessories and clothes. She goes out with friends almost everyday. She sleeps less than usual and tends to fill her day with numerous activities. 4 years back she was on medication for depression.

Cyclothymia

Milder form of bipolar disorder fluctuating between mild depressive and hypomanic symptoms for ≥ 2 years

Lithium Toxicity

Lithium toxicity

- Coarse tremor
- Nausea and vomiting
- Drowsiness
- Hyperreflexia
- Seizure and coma

Diuretics and NSAIDs can cause lithium toxicity by increasing renal absorption

Management

- **Mild to moderate** - IV fluids and stop drug immediately
- **Severe toxicity** - Haemodialysis

Example

A 40 year old man was brought to the hospital with complaints of tremors, vomiting and confusion. He is a known case of bipolar disorder on medication. Recently, he started using ibuprofen for knee pain. Most appropriate test → Serum lithium concentration

Lithium should not be used in pregnancy as it can cause **Ebstein's anomaly**

Delusion

False belief which is firmly sustained and based on incorrect inference about reality

Delusional jealousy (Othello's syndrome) -
Eg, believing a partner is being unfaithful

Grandiose delusion - belief of exaggerated self-worth

Unilateral neglect - belief that one limb or side does not exist

Delusion of Guilt

Eg: I am responsible for the hurricane / flood and thus need to be punished

Persecutory Delusion

Eg: They are spying on me, they are plotting against me

Ekbom's syndrome

Eg: I am infested by parasites

Delusion

Erotomania - Delusional belief that a person of higher social status falls in love with him/her

Folie a deux (shared psychosis)

Delusional belief and even hallucinations are shared between two individuals who are in a relationship

Delusional misidentification

Capgras syndrome:

Fixed belief that someone they know or recognize has been replaced by an imposter

Fregoli delusion:

Delusional belief that different people (More than one) are in fact a single person who changes appearance

Nihilistic delusion

Delusion that the person is already dead, do not exist, are putrefying, or have lost their blood or internal organs

Thought Disorder - Delusion

(Seen in schizophrenics)

Thought insertion

Thoughts are being placed into the patient's head from outside

Thought withdrawal

Thoughts have been stolen from his/her mind

Thought broadcasting

Thoughts are being accessible directly to others

Thought blocking

Sudden break in the chain of thoughts

Eg: The patient stops talking for a few seconds followed by speech in a different topic

Hallucination

Sensory perception which is experienced despite there being no external stimulus

Auditory hallucinations:

- **1st person:**
Voice is heard saying “I am walking”
- **2nd person:**
Voice is heard saying “You are walking”
- **3rd person:**
Voice is heard saying “He/she is walking”

Thought echo

Hallucination of hearing aloud his/her own thoughts a short time after thinking them

Visual hallucination

Eg. Seeing a snake when there is nothing

Note: Seeing a rope as snake is an illusion

Schizophrenia

Diagnosis requires ≥ 2 of the following symptoms:

- Delusions
- Hallucinations, often auditory
- Disorganized speech
- Disorganized behavior
- Negative symptoms - flat affect, social withdrawal

Symptoms should be present for at least 6 months for diagnosis

Management

- Atypical antipsychotics
 - Risperidone
 - Olanzapine
- Depot injections if there is non-compliance
- ECT (treatment resistance)

Schizophrenia

Side effects of antipsychotics

- Weight gain
- Akathisia
- Dystonia
- Tardive dyskinesia (Long term intake)
- Drug-induced Parkinsonism
- Neuroleptic malignant syndrome

Schizoaffective disorder

Schizophrenia + Mood disorder such as depression or bipolar disorder

Schizophreniform disorder:

Symptoms of schizophrenia lasting 1–6 months

Acute Psychosis

Brain trainer:

A schizophrenic man is brought to the ED and is agitated, making threats and is considered a danger to others. How should he be managed?

→ IM Haloperidol

Acute Psychosis

Brain trainer:

When a person has acute psychosis and is either elderly or has Parkinson's disease, what is the most appropriate medication to administer?

→ **IM Lorazepam**

Incongruent Affect

Brain trainer:

A schizophrenic man is seen laughing and smiling about his father's death whom he liked, loved and respected. What term describes his body language?

→ Incongruent affect

Clang Association

Brain trainer:

A schizophrenic says the following. “Life is unfair, I eat air, lawn chair, I like fairs, fairs have food, it must be good, in adulthood, I misunderstood”. What term describes this speech?

→ **Clang association**

Pressure of Speech

Brain trainer:

A 52 year old woman speaks rapidly without any pause and ignores interruptions. She barely even pauses to take enough breaths. What is the SINGLE best term to describe this kind of speech?

→ **Pressure of speech**

Panic Disorder

Presentation

- Periods of intense fear usually for 10-20 min associated with:
 - Palpitations
 - Sweating
 - Tremors
 - Difficulty breathing
 - Chest pain
 - Numbness or tingling sensation
(Hyperventilation → CO₂ washout → Respiratory alkalosis → Hypocalcaemia)

Diagnosis > 1 panic attack with ≥ 1 month of ≥ 1 of the following:

- Persistent concern of additional attacks
- Worrying about consequences of attack
- Behavioral change related to attacks

Management

- **Acute:** Breathing through the bag
- **Long term:**
 - CBT
 - Antidepressants (SSRI)

Generalised Anxiety Disorder

Excessive anxiety and worry for at least **six months**, about a wide range of events or activities

Symptoms

- Palpitations
- Trembling or shaking
- Constant worrying
- Sleep disturbance
- Irritability

Management

- Cognitive behavioral therapy
- SSRIs or venlafaxine (first line)

Social Anxiety Disorder

Presentation

Anxiety causing patients try to avoid situations like:

- Meetings
- Group events
- Eating in restaurants
- Public speaking

Management

- Cognitive behavioral therapy
- SSRIs

Obsessive Compulsive Disorder

Obsessions are unwanted intrusive thoughts, images or urges that repeatedly enter the person's mind. Eg. pathological doubt

Compulsions are repetitive behaviours or mental acts that the person feels driven to perform. Eg. checking a door is locked

Examples

- **Fear of contamination** - Repetitive hand washing
- **Need for symmetry** - Repetitive arranging things
- **Superstitious obsessions** - Repeating activities an exact number of times

Management

- Cognitive behavioral therapy - exposure and response prevention (first line)
- SSRIs

Post Traumatic Stress Disorder

Presentation

- **Re-experiencing:**
 - Flashbacks of the traumatic event
 - Repetitive Nightmares
- **Avoidance:**
 - Avoiding situations and people associated with the traumatic event
- **Hyperarousal:**
 - Hypervigilance for threat
 - Exaggerated startle response
 - Difficulty sleeping

Management

- **Trauma-focused CBT (first line)**
- **Eye Movement Desensitisation and Reprocessing (EMDR)**
- **Stress management**
- **Paroxetine and mirtazapine (second line)**

Phobia

Strong fear or dread of a thing or event, which is out of proportion to the reality of the situation

Agoraphobia

Fear of public and open places

- Fear of travelling in buses
- Fear of visiting a shopping mall
- Fear of even leaving the house (extreme)

Claustrophobia

Fear of confined spaces or of being trapped

Management

- Graded exposure to reduce fear response

Risk Factors For Suicide

- Previous suicide attempts and self harm **(highest risk)**
- Male gender
- Unemployment
- Mental illness such as depression
- Alcohol and drug abuse

Compulsory Admission

Brain trainer:

Under what circumstances may a patient be sectioned (i.e. be admitted to hospital and treated without his/her consent) ?

- Suffering from a mental disorder; and
- Threat to themselves or others

Alcohol Dependence

Brain trainer:

What three questions are highly correlated with alcohol dependence?

- ➔ **How often during the last year have you found that you were not able to stop drinking once you had started?**
- ➔ **How often during the last year have you failed to do what was normally expected from you because of drinking?**
- ➔ **How often during the last year have you needed an alcoholic drink in the morning to get yourself going after a heavy drinking session?**

Developmental Disorders

Tourette's syndrome

- Repetitive tics - Both motor and verbal
- **Examples:**
 - **Motor:** Eye twitching or blinking
 - **Vocal tics:** Throat clearing, grunting and barking
- **Coprolalia** - compulsory saying of dirty words (pathognomonic)
- **Echolalia** - involuntary copying of others' words

Rett syndrome

- Normal development until 6-18 months of age followed by regression of motor, social, and language skills
- Hypotonia with hand-wringing
- Loss of eye contact
- Abnormal, stereotyped midline hand movements

Antipsychotics Side Effects

Neuroleptic Malignant Syndrome

Presentation

- Hyperthermia
- Muscular rigidity
- Autonomic instability
- Due to potent antipsychotics such as **haloperidol** and **fluphenazine**

Management

- Benzodiazepines
- Dantrolene sodium (muscle relaxant)

Acute Dystonia

- Sudden onset of muscle spasms, stiffness, and/or oculogyric crisis
- **Treatment:**
 - Benztropine or diphenhydramine

Akathisia

- Feeling of restlessness and inability to remain still
- **Treatment:**
 - β -blockers, benztropine, and benzodiazepines

Antipsychotics Side Effects

Tardive Dyskinesia

- Involuntary, repetitive body movements, which may include grimacing, sticking out the tongue, or smacking the lips
- Due to chronic use of either typical antipsychotics or metoclopramide

Treatment

- Atypical antipsychotics

Drug-induced Parkinsonism

- Bradykinesia
- Resting tremor
- Rigidity

Treatment

- Benztropine
- Amantadine

Personality Disorder

Borderline personality disorder

- Unstable personal relationships
- Mood swings
- Marked impulsivity
- Self-harm
- Dramatic and attention seekers

Antisocial personality disorder

- Criminal acts - does not follow the law
- Impulsiveness
- Aggressiveness - repeated physical fights
- Reckless and consistent irresponsibility
- Lack of remorse